

How to Transform This From C+ to A-

MAJOR REVISIONS NEEDED

1. THEORETICAL DEVELOPMENT (Most Critical)

Current:

"Financial distress needs redefinition" (stated repeatedly, never delivered)

Needed:

Step 1: Define "mission-critical distress" for public hospitals → Inability to provide safe, accessible care → Workforce exodus → Quality deterioration Step 2: Develop theoretical model → Soft budget constraint framework → Multi-stakeholder distress distribution → Government intervention triggers Step 3: Derive testable predictions → When does transferred distress become unsustainable? → What metrics predict government intervention?

Theoretical Frameworks to Apply:

- Kornai's soft budget constraint theory
- Agency theory (principal = state/citizens; agent = hospital managers)
- Pecking order theory (why hospitals use subsidies not debt)
- Real options (government's options to restructure/close/privatize)

